

Medexa Knowledge Base

Billing Rules, Modifier Usage, Therapy Caps & Common Definitions

Covers Part IV.4 (Modifier Recommendation), Part V.7–9 (AMA Coding Guidance, Time-Based Billing, Therapy Thresholds), and supporting Part X glossary entries.

1. Billing Rules: The 8-Minute Rule & the AMA Rule

Therapy billing under Medicare Part B (and most payers that follow CMS rules) is built around timed and untimed CPT codes. Timed codes are billed in 15-minute units, and two different — and frequently confused — methodologies exist for converting total treatment minutes into billable units.

1.1 The Medicare 8-Minute Rule (CMS Rule)

- Applies to Medicare Part B claims and most Medicare Advantage / Medicaid plans that adopt CMS billing policy.
- To bill one unit of a timed code, the clinician must provide the service for at least 8 minutes.
- Units are calculated on the total timed minutes for the visit (summed across all timed codes), not per-code in isolation.

Standard conversion table:

Total Timed Minutes	Billable Units
8–22 minutes	1 unit
23–37 minutes	2 units
38–52 minutes	3 units
53–67 minutes	4 units
68–82 minutes	5 units
83–97 minutes	6 units

Pattern: each additional unit requires 15 more minutes, with an 8-minute "midpoint" cutoff to round up.

- Remainder minutes below 8 are not billed and are not carried to the next code.
- Untimed (service-based) codes — e.g., PT/OT evaluations (97161–97168) — are billed once per session regardless of time spent, and are not part of the 8-minute calculation.

1.2 The AMA Rule (Substantial Portion Methodology / "Rule of Eights" — CPT Rule)

- Published by the American Medical Association in the CPT manual; used by most commercial/private payers that do not follow CMS's 8-minute methodology.
- Key difference: minutes are calculated per individual code, not pooled across the whole visit.

- A unit is billable once the clinician has performed more than half (a "substantial portion") of the 15-minute increment for that specific code — effectively requiring 8+ minutes for the first unit of each code, but the remainder-pooling logic across codes differs from CMS's method.

AMA's official time table:

Units	Time Range
1 unit	8–22 minutes
2 units	23–37 minutes
3 units	38–52 minutes
4 units	53–67 minutes

Practical distinction that matters for Medexa's rule engine: when a session includes multiple different timed codes with small leftover minutes each, CMS's "sum-then-round" approach and AMA's "round-then-sum" approach can produce different total billable units for the same documented minutes. The billing engine must know which methodology the payer follows before calculating units — defaulting to CMS logic for Medicare/Medicaid and flagging commercial payers for AMA logic (or payer-specific policy, since many commercial payers publish their own variant).

1.3 Implementation Notes for Medexa's Rule Engine

- Payer type (Medicare/Medicaid vs. commercial) should drive which methodology is applied automatically.
- The engine should flag sessions where total documented minutes fall below the 8-minute minimum for any billable unit (undercoding/non-billable risk).
- The engine should flag documentation that doesn't clearly break out minutes per CPT code, since both methodologies require code-level time to be defensible under audit.

2. Modifier Usage

Modifiers refine how a service was performed without changing its fundamental definition. Below are the modifier categories most relevant to PT/OT/SLP billing.

2.1 Discipline (Always-Required) Modifiers

Modifier	Meaning	Usage
GP	Services delivered under an outpatient Physical Therapy plan of care	Appended to every timed/untimed PT code
GO	Services delivered under an outpatient Occupational Therapy plan of care	Appended to every OT code
GN	Services delivered under an outpatient Speech-Language Pathology plan of care	Appended to every SLP code

These are mandatory on Medicare claims and are used by Medexa's coding engine to route claims to the correct discipline-based edit rules.

2.2 Threshold / Medical Necessity Modifier

Modifier	Meaning	Usage
KX	Attests that services beyond the annual therapy threshold are medically necessary and documentation supports continued treatment	Appended once a patient's incurred expenses cross the CY threshold (see Section 3)

2.3 NCCI Bypass / Distinct Service Modifiers

Used to bypass National Correct Coding Initiative (NCCI) edits when two codes that would normally bundle were, in fact, clinically distinct.

Modifier	Meaning
59	Distinct Procedural Service — legacy, catch-all modifier; CMS prefers the more specific X{EPSU} set where applicable
XE	Separate Encounter
XP	Separate Practitioner
XS	Separate Structure/Site
XU	Unusual Non-Overlapping Service

Medexa's NCCI validation layer should prefer suggesting an X-modifier over 59 whenever the documentation supports a more specific reason, since CMS guidance favors specificity.

2.4 Telehealth / Service Delivery Modifiers

Modifier	Meaning
95	Synchronous telemedicine service via real-time audio/video
GT	Via interactive audio/video (legacy, still used by some payers)
93	Synchronous telemedicine via audio-only

2.5 Assistant-Provided Services (Therapy Assistant Modifiers)

Modifier	Meaning
CQ	Outpatient PT services furnished in whole or in part by a Physical Therapist Assistant (PTA)
CO	Outpatient OT services furnished in whole or in part by an Occupational Therapy Assistant (OTA)

- Required when an assistant furnishes more than 10% of a timed service (the de minimis standard).
- These claims are reimbursed at 85% of the standard fee schedule rate under current Medicare policy.
- Medexa's engine should flag any session log showing assistant involvement over the 10% threshold that is missing CQ/CO.

2.6 Repeat / Distinct Visit Modifiers (facility/outpatient)

Modifier	Meaning
76	Repeat procedure by same provider
77	Repeat procedure by another provider

2.7 General Modifier Best Practices for the Rule Engine

- Never stack a discipline modifier (GP/GO/GN) with the wrong discipline's code — this is a common source of denials that the CPT-to-modifier mapping should catch automatically.
- Modifier order matters for clean-claim formatting: pricing modifiers (e.g., CQ/CO) typically precede informational modifiers.
- Every suggested modifier should carry an explainability trace (Part IV.6) showing which documentation elements justified it, since modifiers are a frequent audit trigger.

3. Therapy Caps / Thresholds (CY 2026 CMS Figures)

The original "hard" Medicare therapy cap was repealed by the Bipartisan Budget Act of 2018. It was replaced by two separate dollar thresholds that reset every January 1.

3.1 KX Modifier Threshold (Soft Cap)

- CY 2026: \$2,480 — applies to PT and SLP services combined (one shared bucket).
- CY 2026: \$2,480 — applies separately to OT services (its own bucket).
- These amounts are indexed annually by the Medicare Economic Index (MEI).
- Once a patient's incurred (allowed) charges reach this threshold, every subsequent claim must carry the KX modifier to attest medical necessity, or the excess will be denied.
- The claim that pushes the patient exactly to the threshold does not itself require KX — only claims after it.

3.2 Targeted Medical Review (MR) Threshold

- \$3,000, held flat through CY 2028, at which point it resumes MEI indexing.
- One MR threshold for PT/SLP combined, a separate one for OT.
- Crossing this threshold does not trigger automatic denial — it makes the patient's claims eligible for targeted (not universal) medical review by MACs, based on risk patterns (e.g., billing outliers, prior denial history), not a blanket audit of every claim.

3.3 Practical Rules for Medexa's Threshold Engine

- Track cumulative allowed charges per beneficiary, per calendar year, separately for the PT/SLP bucket and the OT bucket.
- Reset all counters on January 1.
- Auto-flag/prompt for KX once a patient crosses \$2,480 in either bucket.
- Auto-flag an elevated audit-risk warning (not a denial) once a patient crosses \$3,000.
- KX should only be suggested when documentation actually supports continued medical necessity — the modifier is an attestation, not a formality, and misuse is a compliance risk.

- Multiple Procedure Payment Reduction (MPPR): a 50% payment reduction applies to the practice-expense portion of the second and subsequent "always therapy" timed procedures performed for the same patient on the same day (highest-RVU code paid at 100%, remaining codes at 50%). This is separate from the threshold logic but affects the same claims and should be modeled in reimbursement estimates.

4. Common Definitions (Glossary Entries for Part X)

Term	Definition
8-Minute Rule	CMS billing methodology requiring at least 8 minutes of a timed service to bill one unit, calculated on total session minutes across all timed codes.
AMA Rule (Rule of Eights)	CPT-manual billing methodology used by many commercial payers, calculating billable units per individual code rather than pooling minutes across the visit.
Timed Code	A CPT code billed in defined time increments (typically 15 minutes), where reimbursement depends on documented treatment minutes (e.g., therapeutic exercise 97110).
Untimed/Service-Based Code	A CPT code billed once per encounter regardless of time spent (e.g., PT/OT/SLP evaluations).
KX Modifier	Modifier attesting that therapy services beyond the annual threshold are medically necessary, supported by documentation.
Therapy Threshold ("Therapy Cap")	Annual dollar amount of allowed charges per beneficiary above which the KX modifier is required; the original hard cap was repealed in 2018.
Targeted Medical Review (MR) Threshold	A higher dollar threshold (\$3,000 for CY 2026) above which claims become eligible for selective, risk-based review by Medicare contractors.
NCCI (National Correct Coding Initiative)	CMS edit set that prevents improper payment for code pairs that should not normally be billed together.
MUE (Medically Unlikely Edit)	A per-code maximum unit limit CMS considers medically plausible for a single patient on a single date of service.
LCD (Local Coverage Determination)	Coverage policy issued by a Medicare Administrative Contractor (MAC) defining when a service is considered reasonable and necessary in that jurisdiction.
NCD (National Coverage Determination)	Nationwide CMS coverage policy defining whether/when a service is covered under Medicare.
Modifier	A two-character code appended to a CPT/HCPCS code to indicate the service was altered in some specific way without changing its core definition.
GP / GO / GN Modifiers	Discipline modifiers indicating a service was furnished under a PT (GP), OT (GO), or SLP (GN) plan of care.
CQ / CO Modifiers	Modifiers indicating a service was furnished in whole or part by a PT Assistant (CQ) or OT Assistant (CO), triggering an 85% payment rate.

Term	Definition
De Minimis Standard	The 10% threshold above which assistant involvement in a timed service requires the CQ/CO modifier.
MPPR (Multiple Procedure Payment Reduction)	A 50% reduction to the practice-expense portion of payment for the second and subsequent "always therapy" procedures billed for one patient on one day.
Plan of Care (POC)	The certified treatment plan (diagnosis, goals, frequency, duration) required before ongoing therapy services can be billed.
Medical Necessity	The clinical justification, supported by documentation, that a service is appropriate for the patient's diagnosis, condition, and functional deficits.
Medicare Economic Index (MEI)	The annual inflation-style index CMS uses to update the therapy thresholds and other payment figures.
Substantial Portion	Under the AMA Rule, the requirement that more than half of a 15-minute increment be furnished before billing a unit for that code.

Sources: CMS Medicare Claims Processing Manual, Pub. 100-04, Ch. 5 §10; CMS CY 2026 Physician Fee Schedule Final Rule; CMS Therapy Services webpage (KX modifier thresholds, updated for CY 2026); APTA payment policy guidance. Dollar thresholds reflect CY 2026 figures and should be re-verified each January when CMS issues updated amounts.